

Payment Integrity Retrievals

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Priority	Must Do (Tier 1) ▾	Vertical Finance	Frank Goehmann	10/3/2025
Financial Model Link	 FY26_Payment ...	Other owner(s)	John Welch Kelly Taylor Robert Connely Kristyn Day	10/3/2025 <sign off date> 10/3/25 10/2/25

1. Executive Summary

Datavant has the opportunity to expand its payer solutions portfolio by enabling clinical data acquisition for Payment Integrity programs. This initiative addresses payer demand for faster, higher-quality access to clinical documentation that underpins accurate claims adjudication. By leveraging Datavant's embedded digital network, retrieval capabilities, and scalable data infrastructure, we can provide differentiated solutions to ensure claims are paid accurately, reduce overpayments, and improve customer outcomes. **We will aim to deliver an MVP solution by the end of Q1, with a goal of capturing \$9.2 net revenue(Payer revenue – lost Provider revenue) by the end of 2026.**

2. Problem Statement & Vision

Problem Statement

Despite being a critical and contractual touchpoint between Payers and Providers, Datavant does not currently have a solution to support record retrieval for Payment Integrity programs.

Vision Statement

Datavant's Medical Record Retrieval for Payment Integrity solves fragmented clinical data acquisition for Payers by providing a **digital-first, secure, auditable, and scalable record retrieval solution** that unlocks Datavant's proprietary digital network for this use case, reduces administrative burden for Providers, and enables visibility for both Providers and Payers.

2a. Market & Business Context

Business Context

The payment integrity industry focuses on reducing fraud, waste, and abuse in U.S. healthcare payments by ensuring accuracy, compliance, and legitimacy in claims. Traditionally divided into Clinical and Non-Clinical categories, this discussion centers on Clinical Payment Integrity, which can involve pre-payment reviews (typically resolved within 30 days per state guidelines) or post-payment reviews (up to two years). To validate claims, payers and vendors require timely access to clinical records and itemized bills, but current retrieval models are slow, manual, and inconsistent, often relying on provider HIM departments or offices to fulfill requests under regulatory and contractual requirements. Without scalable, digital-first solutions, payers face inefficiencies and missed savings opportunities, while providers shoulder operational burdens and limited visibility.

Although payment integrity is a cornerstone of payer-provider interactions, it is also a frequent source of tension. Providers may perceive payers as using medical record reviews to deny claims unnecessarily, creating mistrust. It's critical to note that Payment Integrity is going to continue to grow in volume, so Providers will continue to receive these requests regardless of our role in it. We have the opportunity, as a neutral, trusted arbiter, to reduce abrasion. As a health data platform serving both payers and providers, it is essential that solutions emphasize transparency, reliability, and trust to reduce friction and create a foundation for collaborative, value-driven payment integrity practices.

Market Trends

- **Payment Integrity represents a \$1.58B market in 2025, and is projected to grow to approximately \$14.12B by 2034 (27.5% CAGR) as Payers face increasing regulatory and financial pressure to prevent improper payments; Payers currently represent 35.50% of Payment Integrity end-user market share compared to 64.50% represented by Providers, however the Payer segment is anticipated to grow at the highest rate from 2025 to 2034.¹**
- **Post-Payment Integrity programs represented 49.6% of the U.S. Payment Integrity Program market share in 2024, with Pre-Payment representing 25.60%, however, Pre-Payment is expected to grow at the fastest rate from 2025-2034.**
- **Clinical reviews are among the most common use cases, and within Clinical Reviews, Inpatient DRG audits are one of the most common Payment Integrity programs used by Payers today.**
- **The value a Payer gets from Payment Integrity programs varies by sources, but a Payer could see anywhere from 10-25% in savings per claim.²**

¹ [Precedence Research – U.S. Payment Integrity Market Size and Forecast 2025 to 2034](#)

² [Apixio Payment Integrity Brochure](#)

- Datavant manually fulfilled ~1.8M DRG/Payment Integrity record requests from May 2024–April 2025 through PayI³.

Competitive Landscape

- The Payment Integrity ecosystem includes major players like Cotiviti, Optum, MultiPlan, Machinify, EXL, and several niche vendors. These companies support payers with solutions such as data mining, AI, clinical reviews, reporting, and risk-sharing programs, while a few payers like Humana have built in-house capabilities.
- Some vendors have invested in direct digital connections with providers for record retrieval, but most still rely—directly or indirectly—on Datavant (via PayI) or its competitors for this function.
- **Differentiation opportunity:** Datavant’s proprietary digitized embedded network and clinical data acquisition platform can allow customers to seamlessly integrate and retrieve complete and accurate medical records while maintaining trust and visibility with Providers.

Opportunity Sizing

- [Strat Plan 2026: Payment Integrity TAM Calculations](#)
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3. Recommended Approach

Initial Focus in 2025

Launch with support for Post-Pay Inpatient DRG reviews, because it is the most common Payment Integrity review and the review timelines allow for Datavant to meet TAT guidelines successfully. We will leverage the Provider Locator capabilities on Switchboard once that goes live in Q2 2026. We will also be developing a shared capability with an external Embedded Network pre-match API.

Product Construct:

- This offering will be **request-based** (ad hoc or batch) and not project-based
- Datavant will **charge per successfully retrieved record** and not be penalized for unsuccessful attempts if response SLAs are met
- This product is **currently scoped to be an Embedded-only (both digital and manual) product**; DV is incentivized to grow its digital network that supports Payment Integrity request types.
- For an early MVP, a lean customer success team can support customers in facilitating the initial Embedded network prematch and initiating requests – this will expedite go-live timelines with customers ([see memo for more detail on product requirements](#))
- **For Provider Vertical:**

³ [Raw Data, BI Dashboard](#)

- **Provider-Facing Analytics:** Surface analytics on Audapro to provide real-time visibility on Payment Integrity requests as a selling point
 - **Healthsource Fulfillment for Payment Integrity:** Support Payment Integrity request and due dates for manual fulfillment
 - **Future Exploration/ Expansion:**
 - **Itemized Bills:** Investigate opportunities to retrieve Itemized bills from DV's Embedded network in order to expand beyond DRG/Audit Payment Integrity programs with support for itemized bills and pre-pay.
 - **Enable adding additional sources:** Explore additional retrieval sources through digital affiliates and Request Manager
 - **Key Assumptions:**
 - Payers and Payment Integrity vendors will adopt digital-first solution with <30 TAT
 - Datavant will have a sizeable volume of charts through Embedded network that consists of Providers who have opted in for digital payment integrity retrieval
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3a. Operational Considerations

Payer Vertical

- ***New* Payment Integrity Retrieval Operations:** Lean team of digital-savvy operators that can interface with customers to map data, set up Payment Integrity-specific request templates, execute pre-match and retrieval (if API connection is not established), triage data issues, and monitor/QA performance at customer and request levels
 - **Training and Quality monitoring:** Build internal expertise in Payment Integrity templates, workflows, and KPIs
 - **Operational KPIs & Controls:** Define and monitor customer and PI KPIs and controls

Provider Vertical

- **Provider Digital Operations:** Configuration of Payment Integrity digital request type for target Providers in Embedded digital network

3b. Go-to-Market Considerations

Payer Vertical

- **Sales Assumptions:** [See Case](#)
- **Business Development Pipeline:** Develop sales pipeline and target prospective customers for Payment Integrity solution
- **Sales Enablement Training:** Equip sales teams with PI-specific case studies, ROI benchmarks, structured SOWs and positioning.
- **Develop Co-Commercialization Strategy & B2B Product marketing:** Identify channel partner growth strategy with Payment Integrity vendors

- **Engagement Model / Account Management:** Define operating model for managing Payment Integrity accounts, inclusive of both Payers and Payment Integrity vendors.
 - Develop Playbook: Define playbook of metrics and engagement to drive growth with customers

Provider Vertical

- **Digitization Pipeline:** Develop engagement pipeline and target prospective customers for Payment Integrity solution

3c. Legal, Compliance & Security Considerations

- **Legal:** Support in developing Payment Integrity Product SOWs and updating contracts with Embedded Providers with opt-in

4. Resource & Investment Overview

(In \$ Millions)	2026 Q1	2026 Q2	2026 Q3	2026 Q4	2026 Total	2027 Total	2028 Total	3Y Total
FTE Headcount Costs	\$0.73	\$0.79	\$0.78	\$0.78	\$3.08	\$3.02	\$3.14	\$9.23
Contractor Costs					0			0
Non-HC Vendor Costs					0			0
Total	\$0.73	\$0.79	\$0.78	\$0.78	\$3.08	\$3.02	\$3.14	\$9.23

Section 3: Projected Financial Returns

(In \$ Millions)	2026 Q1	2026 Q2	2026 Q3	2026 Q4	2026 Total	2027 Total	2028 Total	3Y Total
Incremental Revenue	\$0.00	\$4.17	\$4.21	\$4.28	\$12.66	\$30.48	\$39.32	\$82.45
Incremental Gross Profit	\$0.00	\$2.98	\$3.06	\$3.16	\$9.21	\$22.89	\$30.00	\$62.09
Cost Out (OPEX)					\$0.00			\$0.00
Incremental EBITDA	\$0.00	\$2.98	\$3.06	\$3.16	\$9.21	\$22.89	\$30.00	\$62.09

3-Year ROI 5.73

4a. Staffing Considerations

- **Product & Tech:** We will spin up a new pod to support Payment Integrity staffed by a combination of new and existing headcount. We will need 1 new PM FTE, 0.5 Design FTE, 5 new Eng FTE on Payer, 0.5 new Eng FTE on Provider Connections, 2 new Eng FTE on Healthsource ROI and 1 New BI&A. We will also leverage capacity from Data Foundations and Payer Pipelines.

4b. Dependencies

Dependency	Accountable Team	Resourcing	Signoff
Provider opt-in for digital Payment Integrity retrieval	Provider Account Management	No Additional	

Payment Integrity digital configuration	Provider DigOps	2 FTE	
Biz Dev/Sales pipeline	Payer Growth	Captured in investment	Complete
PI Customer account management and retrieval Operations	Payer DigOps	Captured in investment	Complete
Healthsource support for Payment Integrity Request Type, TAT and approval queue	ROI – Healthsource	2 Eng FTE 0.25 Design FTE 0.25 PM FTE	Complete
Scaling Pre-Match support for Payment Integrity use case and external API	Data Foundation	Already captured	Complete
Provider Connections support	Provider Connections	0.5 Eng FTE	
BI&A support for new product	BI&A	1 BI&A	

5. Metrics for Success

KPI	Baseline	Target
# Digital Charts Available for Payment Integrity	<<X%>>	<<Y%>>
Avg. Turnaround Time for Retrieval	30 days	≤ 15 days
Retrieval Yield from Digital Connections for Payment Integrity	<<X%>>	<<Y%>>
% Positive Network Requests (<i># of requests for PI retrieval made for pre-match that resulted in yes / # of total PI requests for retrieval</i>)	0	TBD
Customer Adoption	0 customers	X customers by EOY26
Revenue Contribution	\$0	TBD 2026 in-year
Average Billable RPC	N/A	TBD

6. Risks & Mitigations

Risks	Mitigations
Provider resistance to Payment Integrity retrieval requests	Early provider engagement with investment in training and enablement for Provider growth team; Explore “carrots” such as rev share opportunities
Low digital yield in early phases → lower revenue	Target top 25 Providers by volume for opt-in and configuration by EoQ1 2026; consider interim manual fallback
Customer acquisition and adoption slower than expected	Invest in higher digital yield in first half of year; anchor with Machinify & Optum
Payers not willing to pay for charts (channel partners appear willing to do so)	Focus on those willing to pay (we have history of channel partner willingness to pay; we have heard ShareCare charges \$35–40/chart successfully)

7. Links & Supporting Materials

- [Briefing Memo: Clinical Data Acquisition Use Cases for Payment Integrity](#) (Feb 2025)
- [Memo: Operationalizing Record Retrieval for Payment Integrity](#) (Jul 2025)